



Electroconvulsive Therapy (ECT) via Thymatron IV[®] Management Clinical Practice Standard

Scope

Site	Department, division or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	Mental Health	All Staff

Key points

- ECT is only administered at Mill Street Campus, Bentley Health Service (BHS).
- The administration of ECT is the responsibility of the treating psychiatrist or medical officer (MO). The procedure is undertaken with assistance from the ECT Nurse, and in collaboration with a trained anaesthetic team.
- Treating Teams are required to complete (in full) the ECT Consent to Treatment Form in collaboration with the patient and their carers or personal support persons (PSPs) and/or consider guardian, or to provide the approved Mental Health (MH) Tribunal letter for involuntary patients.
- Staff involved in ECT must have an awareness of best practice and comply with the requirements of the Chief Psychiatrist (CP):
 - Standards for Clinical Care
 - Chief Psychiatrist's Practice Standards for the Administration of Electroconvulsive Therapy
 - E.C.T. Guide: [The Chief Psychiatrist's Guidelines for the use of Electroconvulsive therapy in Western Australia 2006](#) (direct link)
 - [Charter of Mental Health Care Principles \(Mental Health Act \[MHA\] 2014\)](#) (direct link).
- All emergency ECT must be approved by the CP or the nominated RPBG Psychiatry delegate.
- If patient is travelling from another hospital or service for ECT (e.g. Royal Perth Hospital [RPH] Mental Health Unit [MHU]) the referring ward staff **must arrange an escort nurse** to accompany the patient during the transfer to BHS, treatment, the recovery post ECT and the transfer back to home ward or area.

Electroconvulsive Therapy (ECT) via Thymatron IV[®] Management CPS

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General information

ECT is a medical treatment for numerous psychiatric conditions. The treatment consists of the administration of a muscle relaxant and general anaesthesia prior to the patient receiving an electrical stimulus applied trans-dermally to the scalp in order to induce a grand-mal seizure ¹ [Level GPP].

The number of treatments for ECT is determined on an individual basis. A full course for most patients of ECT is 6-12 treatments given 2 or 3 times per week and may require hospital admission ² [Level GPP]. The patient's treating team is responsible for the overall care of the patient and will plan admissions and ongoing care based on the patient's individual needs. The ECT team is only responsible for providing the ECT treatments.

Electroencephalogram (EEG)

EEG monitoring is a standard that must occur with all ECT treatment in Western Australia ¹ [Level GPP]. EEG monitoring is crucial during ECT as it enables the practitioner to confirm that a cerebral seizure has occurred and ended in a timely fashion ² [Level GPP].

In ECT the Thymatron IV[®] (Figure 2) is used to apply the electrical stimulus as well as monitor seizure parameters such as the EEG and ECG. Functions of the Thymatron IV[®] include:

- seizure induction
- anaesthesia depth monitoring
- electronic medical record keeping
- EEG coherence measures
- EEG amplitude measures
- audible EEG monitor
- continuous digital heart rate measures
- internal memory for stored treatment session
- extended seizure alert
- computer determination and printout of EEG and motor seizure durations

Electrode placement

The electric current used to induce the seizure passes briefly through the brain via two electrodes applied to the scalp. The three most common electrode placements ([Figure 1](#)) are:

- bi-temporal
- right unilateral
- bifrontal ³ [Level GPP].

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General information - Electroencephalogram (EEG) cont'd

Thymatron protocols for titration

During the first ECT treatment, the Treating Doctor is to complete the titration as per the written **Thymatron Protocols for Titration**, which are kept within the ECT Suite next to Thymatron.

- **Note:** Use of multiple ECT (the delivery of more than one adequate seizure per treatment session) is not acceptable as per [Chief Psychiatrist's Practice Standards for the Administration of Electroconvulsive Therapy](#).

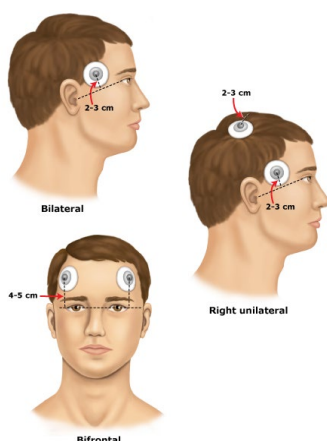


Figure 1: Electrode placement in ECT ⁴



Figure 2: Thymatron IV[®] machine ⁵.

Note: Electromyography (EMG) monitoring is no longer performed as it adds no relevant clinical information.

Main indicators for the use of ECT

^{6,7} [Level I], ⁸ [Level III-2], ¹ [Level GPP]

- Moderate to major depressive episode
- Mania
- Schizophrenia.

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General information cont'd**Contraindications for the use of ECT ^{6,9} [Level I], ¹ [Level GPP]**

There are no absolute contraindications for ECT. For each patient, the risk needs to be weighed against the potential benefits after consultation with the ECT psychiatrist, the treating psychiatrist, the anaesthetists and any other relevant medical specialist (e.g., neurologist for raised intracranial pressure; Ear Nose and Throat (ENT) specialist consulted for cochlear implants). The outcome of these discussions would need to be presented to the patient and their carer/advocate as part of the informed consent process and, in the case of involuntary patients, presented to the MH Review Tribunal as part of the application for involuntary ECT. Ultimately, the decision to proceed with treatment should rest with the ECT psychiatrist and only after the patient has been deemed fit for the procedure by the anaesthetist and approved for treatment at BHS.

Patients with an American Society of Anaesthesiology (ASA) score of 1 - 3 can have ECT at BHS. Patients with an ASA of 4 and above must be referred to a tertiary hospital such as Midland St John of God for treatment.

ECT may be deemed necessary even when 'relative contraindications' are present:

- unstable or severe cardiovascular conditions, such as myocardial infarction, unstable angina, poorly compensated heart failure, and severe valvular cardiac disease including critical aortic stenosis
- past history of aneurysm or vascular malformation that might be susceptible to rupture with increased blood pressure (BP)
- increased intracranial pressure, as may occur with some brain tumours or other space-occupying cerebral lesions
- cerebral infarction
- pulmonary conditions such as severe chronic obstructive pulmonary disease, asthma, or pneumonia
- elevated serum potassium, obesity and venous thrombosis.

Conditions having substantially higher risk with ECT include:

- pheochromocytoma (benign adrenal gland tumour that results in the release of increased adrenaline and noradrenaline)
- retinal detachment
- acute narrow angle glaucoma
- cochlear implants. Consider unilateral treatment on side away from the implant
- pregnancy -adverse events include foetal heart rate reduction, uterine contractions, premature labour and child mortality. ECT during pregnancy is to be considered only as last resort treatment under very stringent diagnostic and clinical indications.

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General information cont'd**Adverse effects and potential complications of ECT**

Potential side effects include ¹⁰ [Level I], ^{11,12} Level III-2], ^{1,2,4,13} [Level GPP]:

- headache: usually mild and responds with minor analgesia such as paracetamol. If headache remains severe after mild analgesia or it persists beyond 12 hours post-treatment, then medical review is required
- myalgia: usually mild and responds with minor analgesia such as paracetamol. If soreness persists, a medical review is required
- drowsiness/agitation: should resolve within 3 hours. If drowsiness persists then a medical review is required
- memory dysfunction/cognitive dysfunction: all reports of memory or cognitive difficulties are to be reported to the treating medical team
- post-ECT delirium: can occur immediately post-ECT. Management of delirium is outlined in the [RPBG Delirium Management CPS](#) and involves identifying and treating the underlying cause

Noting: that for any acute post-ictal delirium, treatment will be provided by the ECT anaesthetic team in the ECT suite as required until the patient has been assessed as being re-orientated to time, place and person

- anterograde amnesia: the decreased ability to retain newly acquired information – usually resolves within 2 weeks after completing the course. Patients should be advised not to drive for 24 hours following treatment or operate machinery ^{1,9} [Level GPP]
- retrograde amnesia: involves forgetting recent memories for weeks to months following ECT treatment and is the most anxiety-producing effect of ECT. Deficits relating to knowledge about public or world events (impersonal memory) are the most noticeable and persistent. ^{1,2} [Level GPP]
- adverse cardiac events: most commonly reported are acute heart failure, arrhythmia, and acute pulmonary oedema. Activate Medical Emergency Team (MET) as clinically indicated and inform anaesthesia team. If the anaesthetist rostered for the ECT list is no longer on site, call RPH Duty Anaesthetist: 0424155037
- treatment emergent manic state
- nausea and vomiting
- fractures/falls (especially for patients 60 years+ and/or with existing comorbid medical conditions)
- soft tissue damage. If suspected requires medical review
- dental damage may occur when the oral bite block is not fully protective. Loose teeth are at risk for dislodgement during the procedure and possible aspiration and should be stabilised or extracted before ECT ² [Level GPP]
- skin burns
- though a rare outcome, ECT may cause death as a result of various complications of ECT such as reactions to anaesthesia, cardiovascular complications, pulmonary complications, or stroke ¹⁴ [Level I].

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Cognitive testing requirements

Prior to, during and at the conclusion of ECT course, the Consultant Psychiatrist or delegated clinician of the treating team is required to complete the following cognitive and orientation screenings¹⁵:

Measure	Frequency during ECT course
Clinical Global Impression scale - Severity (CGI-S)	• Pre-ECT course • Post-ECT course • Every 3 months for patients on maintenance • After last continuation/maintenance ECT treatment
Clinical Global Impression Scale - Improvement (CGI-I)	• Weekly during ECT • Post-ECT course • Every 3 months for patients on maintenance • After last continuation/maintenance ECT treatment
Montreal Cognitive Assessment (MoCA)	• Pre-ECT course • Post-ECT course • Every 3 months for patients on maintenance • After last continuation/maintenance ECT treatment
Brief ECT Cognitive Screen (BECS)	• Pre-ECT course • The day after 3rd or 4th treatment
Orientation (Orientation Questionnaire Form)	• Pre-ECT course • 30 mins after each ECT treatment • Post-ECT course • After last continuation/maintenance treatment
Quality of Life, Enjoyment & Satisfaction, Short Form (Q-LES-Q SF)	• Pre-ECT course • Post-ECT course • Every 3 months for patients on maintenance • After last continuation/maintenance ECT treatment

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ECT referrals

ECT team accepts referrals from Services
- BHS/RPH Inpatient wards/ Consultation Liaison (C-L) Service - Bidi Wungen Kaat Centre (Transitional Care Unit [TCU]) (BHS) - East Metropolitan Health Service (EMHS) Community Mental Health Services (CMHS) - Referrals from other services may be considered on a case by case basis
Referrals can be (in order of preference)
- Initial Phone call to ECT nurse coordinator to discuss referral (contact details as per RPBG ECT Healthpoint page) - Emailed to: BHS.DayTreatmentUnit@health.wa.gov.au - Fax and postal referrals can be arranged on request to the ECT Nurse Coordinator
On receipt of a referral, the ECT Nurse Coordinator will
- Respond to the referrer and provide required documentation and instructions - Upon receipt of the required documentation, contact referrers to arrange ECT commencement and admission requirements.

Referral documentation includes:

- *ECT Consent to Treatment form* or MH Tribunal approval letter
- Relevant MHA 2014 forms
- Pre-ECT Cognitive Testing (ECT Team will advise on required assessments)
- The following Statewide Standardised Clinical Documentation (SSCD)
 - *MH Assessment Form*
 - *Physical Examination Form*
 - *Risk Assessment Management Plan (RAMP)*
- Other specific requirements that a patient may have, e.g., an interpreter or gender/bodily diversity/sexuality preferences/other requirements.

Once the ECT referral is accepted the ECT file tab is added to the patient medical record by the ward clerk or ECT nurse coordinator and the treating team is to:

- complete front sections of the patient's ECT treatment chart
- documentation in the integrated progress notes stating:
 - the current mental status of the patient including mental health legal status under the MHA 2014
 - base line cognitive functioning
 - information about ECT being provided to the patient and/or family/ PSP
 - findings of a physical health assessment and history within 7 days prior to commencing an inpatient course of treatment, and 7 days before the start of an outpatient course of ECT ¹⁶ [Level GPP].

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ECT referrals cont'd

- ensure signed *Consent to Electroconvulsive (ECT) Treatment Form* has been completed:
 - for maintenance ECT consent must be obtained after 12 treatments or every three (3) months, whichever comes first ¹³ [Level GPP]
 - for a course of ECT, the consent is valid only for the documented treatment number (max 12 treatments for one consent) and with the specified electrode placement
 - advise patient of pre-procedural fasting requirements (refer to **Pre- and Post-Procedural Management NPS**). Fasting status is documented on the morning of treatment by nursing staff on the *pre-ECT Treatment Nursing Checklist* ([Appendix I](#))
- ensure *Physical Examination Form* (SSCD) is completed a minimum of 7 days prior to ECT commencement date.
- complete baseline pathology investigations and ECG and document any relevant past medical history within the integrated progress notes. Individual patient assessment will determine need for investigations (e.g. pre-anaesthetic screen). ECT Team will advise of required tests.
- complete all required cognitive and orientation screenings as per the above Cognitive Testing Requirements table.
- any incomplete referral documentation will be flagged for completion by the ECT Coordinator to the treating team prior to the patient commencing their first treatment.

Consent

Policies and procedures relating to consent for ECT are available in ECT suite, wards, hub and relevant orientation manuals. The ECT nurse coordinator checks the *ECT Consent to Treatment Form* or MH Tribunal letter for each patient prior to every ECT treatment to ensure consent for the procedure and the anaesthesia is valid, in date and as per the patient's MHA 2014 status.

The consent requirements for ECT will consider the following:

- legal status of the patient under the MHA 2014
- capacity of the patient to consent
- requirements for informed consent (defined by Part 14, Division 1 of MHA 2014)
- for substitute decision maker guidance on voluntary non-urgent ECT treatment refer to Section A of the *Consent to Electroconvulsive Therapy Treatment Form*.

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Consent cont'd**Voluntary patients**

- All voluntary patients must give consent to treatment via the:
 - *Consent to ECT Treatment Form*
 - includes details of electrode placement. If the ECT Team changes electrode placement during the course of treatment, new consent forms are required
 - *Patient Consent to Anaesthesia General or Regional Form* - obtained by the treating anaesthetist prior to commencing treatment
- Both the *ECT Consent to Treatment Form* and the *Patient Consent to Anaesthesia Form* are valid for up to 12 treatments or 3 months, whichever comes first
- All voluntary consent can be withdrawn at any time during the course of treatment
- ECT on a child under 14 years is prohibited under the MHA 2014¹⁶ [Level GPP]
- The MH Tribunal must provide approval under Part 21, division 6 of MHA 2014 for both voluntary and involuntary patients aged 14 to 18 years. Emergency ECT on any person under the age of 18 is forbidden under the MHA 2014.¹⁶ [Level GPP].

Involuntary patients

ECT treatment can be given without consent of the patient where it is recommended by the treating Psychiatrist to the MH Tribunal and the MH Tribunal approves the treatment prior to intervention commencing in accordance with Part 21, division 6 of MHA 2014. The treatment must be according to the stipulated ECT treatment plan set out in the MH Tribunal application letter (including treatment numbers and specified course duration)
 Note: Carer or PSP to be informed of recommended patient treatment with rationale where possible, prior to commencement.

Emergency ECT

An application for emergency ECT can be made by the psychiatrist when:

- an involuntary patient (for **18 and over ONLY**) is urgently in need of treatment, **and**
- waiting for a MH Tribunal would pose a significant risk to the patient.

All applications for emergency ECT must follow the process below

- a psychiatrist must seek written approval for emergency ECT from the CP or delegate before any treatment is given
- a psychiatrist seeking the CP or delegate's consent to provide emergency ECT must via a telephone call or email provide the following details:
 - the involuntary patient's demographic details
 - the clinical reasons for seeking to treat the patient with emergency ECT, which demonstrates that:
 - it is required to save the patient's life or
 - because there is an imminent risk of the patient behaving in a way that is likely to result in serious injury to the patient or another person
- confirmation that the patient is an adult >18 yrs
- confirmation that a MH Tribunal hearing will be sought at the earliest opportunity
- on receipt of a request the officer exercising the CP's power of delegation must respond in writing via email and copy the email in to: monitoring@ocp.wa.gov.au
- note that the CP or delegate is only authorising one, or in exceptional circumstances, 2 treatments until the MH Tribunal can be convened.

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Consent - Maintenance ECT cont'd**Maintenance ECT**

Ideally all patients receiving maintenance ECT will have ongoing *ECT Consent to Treatment Form* completed by their community treating teams.

- ECT consents are valid for a maximum of 12 treatments or 3 month duration, whichever comes first
- To ensure timely completion, ECT team will communicate with treating teams prior to re-consents being due
- Anaesthesia consents are renewed in-line with ECT consent due dates and are completed prior to treatment with the anaesthetist
- In the event of a patient being unable to complete the ECT consent with the community team, they should contact the ECT nurse coordinator who will arrange completion of the consent when patient presents to hospital on day of treatment.

The ECT team will provide ongoing communication about the patient's ECT treatment or maintenance ECT to the outpatient referring service via email.

- Before beginning a maintenance series of ECT, the treating psychiatrist should ensure that the patient's integrated notes include the following documentation:
 - rationale for maintenance ECT
 - review of clinical response, side effect
- Request an anaesthesia consultation/evaluation during maintenance ECT, when there is a significant change in the patient's medical status or medications
- Treating Psychiatrist should enter the following information in patient's integrated notes:
 - a plan for post-ECT clinical management/treatment frequency/course duration/ follow-up of adverse effects.

Patient information requirements

The referring doctor is required to:

- inform the patient, carer, PSP of planned duration of ECT course, agreed electrode placement and probable length of stay in hospital
- provide patient with [Mental Health Commission's Consumer Handbook to the Mental Health Act 2014](#) (sections 7 & 8) as well as relevant information pertaining to treatment
- provide the BHS patient and carer/PSP information leaflet (available in other languages if required) and to offer the patient the ECT patient diary (for outpatients)
 - patient information is displayed in prominent position/s in the ECT suite
- consider the needs of patients, their families, carers or PSPs including cultural and linguistic requirements in providing ECT treatment that is accessible and responsive to their specific needs
- inform the patient of their right to have their nominated carer involved in the management of ECT treatment and in all aspects of their care needs.

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ECT team roles and responsibilities

Medical and nursing staff are individually responsible for maintaining the appropriate levels of accredited training/competency in ECT treatment. The minimum staffing in the ECT suite is to include:

- ECT Psychiatrist
- Anaesthetist
- Anaesthetic technician
- ECT nurse coordinator
- ECT recovery nurse
- ECT escort nurse with each patient.

ECT nurse coordinator

- Is a Clinical Nurse (CN), Level 2, registered with Australian Health Practitioners Regulation Agency (AHPRA)
- Demonstrate clinical experience and competence specific to the role of ECT
 - Have completed or be working towards completing the WA ECT Training Program (run by Perth Clinic) and/or optional- ECT Advanced Course (Wesley Hospital, Kogarah, NSW)
 - Attended an airway management (basic skills) course (provided by BHS Nurse Educators) to enable safe recovery of patients following ECT
 - Provide regular sessions of ECT escort nurse training to hospital nursing staff (these are not a mandatory requirement for the escort nurse role, but recommended)
- In collaboration with the ECT medical lead, the ECT nurse coordinator oversees the supervision, organisation and planning of all aspects of ECT delivery:
 - ensuring stock and equipment used in the ECT Rooms complies with safety, servicing and product testing requirements at all times, including undertaking equipment audits compliance
 - receiving referrals and liaising with the ECT treating team to ensure appropriate referrals are booked and relevant documentation is completed prior to treatment commencing
 - liaising with EMHS MH Patient Flow Coordinator prior to ECT treatment days to ensure allocation of beds and recovery wards for all listed outpatients receiving ECT
 - ensuring ECT processes are followed appropriately and evaluation and reporting requirements to the Office of the Chief Psychiatrist (OCP) are met
 - ensuring annual ECT training records are provided to the OCP
 - liaising with internal and external agencies that oversee the care of their patients (pre- or post-procedure)
- Development and evaluation of appropriate policies/procedures to guide ECT service and nursing activities
- Provides education and support for other nursing staff working within the ECT suite to ensure safe patient centred care
- Maintains a register of ECT statistics for OCP reporting and auditing purposes.

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ECT team roles and responsibilities cont'd**ECT medical lead**

ECT services are administered by a Consultant Psychiatrist who has completed an approved training course and is credentialed to provide ECT.

Responsibilities include:

- ensure that there is a functioning credentialing and clinical privileging system for ECT, and that training and competency requirements consistent with these guidelines are established and maintained. This is recorded in a training log on the W drive as part of medical credentialing and is stored by the ECT nurse coordinator and senior medical employment officer
- Assess by direct supervision the working knowledge and competency of the medical staff in charge of the treatment and the ECT nurse in the use of the Thymatron IV®
- provide regular ECT medical review sessions for interested medical staff to discuss and review ECT treatment courses and to provide updates on contemporary practice of ECT and relevant guidelines
- regularly attend and participate in at least 12 ECT treatment sessions per year
- liaise with and advise other professionals regarding ECT practice and procedures
- undertake continuing professional development of ECT in relation to knowledge and skills
- report serious incidents in relation to the practice of ECT promptly via Datix Clinical Incident Management System (CIMS) and to the CP
- participate in quality improvement activities relevant to ECT services.

Anaesthetist

The administration of anaesthesia is performed by a Consultant Anaesthetist or an appropriately supervised anaesthesia registrar.

The anaesthetist's responsibilities include to:

- obtain anaesthetic consent and verify appropriate patient fasting status
- provide and document an anaesthesia consultation/review before each ECT treatment, to identify any changes to medical status or medications
- administer and document the anaesthesia medications on the anaesthesia record
- ensure against dental damage with appropriate use of bite block
- pre-operative necessary modification of antidepressant drugs should be discussed with the attending psychiatrist
- participate in sign-in and time-out sections of the ECT procedural safety checklist
- transfer patient to recovery post ECT. All patients are transferred to recovery once assessed as having patent airway
- provide clinical handover to the recovery nurse.

Anaesthesia Technician

- Assisting with procedure under the direction of the anaesthetist
- Check and set up anaesthetic equipment as per roles and responsibilities outlined in the **RBPB Anaesthesia Technicians Professional Standards and Scope of Practice.**

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ECT team roles and responsibilities cont'd**ECT recovery nurse**

- Must be a Registered Nurse (RN), Level 1, trained in the care of patients recovering from anaesthesia and registered with AHPRA
- Works under the direction of the ECT nurse coordinator to provide comprehensive evidence-based nursing care to patients within the ECT setting including management of patients during treatment and in post-ECT recovery
- Attend an airway management (basic skills) course (provided in-house by RPBG clinicians) to facilitate safe recovery of patients following ECT
- Provide emergency assistance to airway and hemodynamically compromised clients in recovery until assistance arrives
- Provide assistance with the running of the ECT Suite on the treatment days.

ECT escort nurse

- Ward staff who arrive to the ECT suite as the patient's escort nurse will be supported by the ECT nurse coordinator and ECT recovery nurse to ensure familiarity with their required roles and responsibilities during ECT treatment and recovery.

Admission procedure for ECT outpatients (day cases)

If patient is travelling from another hospital/ service for same-day ECT (e.g. RPH MHU) the referring ward staff **must arrange an escort nurse** to accompany the patient during the transfer to BHS, treatment, the recovery post ECT and the transfer back to home ward/area. The patient is not required to be admitted at BHS for the day - they remain an inpatient on their home ward.

Outpatients arriving to the ward on the same day for ECT treatment will arrive at an agreed time as discussed with the ECT nurse coordinator to facilitate admission processes. On arrival, ward nursing staff are to check the patient's identity with the details in the medical record folder and apply wristband identification/or other approved method of identification as applicable. Refer to **Identification of Patients SOP**

Admitting nurse is to complete the following:

- Confirm current details on WebPAS (printed by admitting HIMS clerk) match patient's current address and contact details (incl NOK)
- Admission nursing entry in patient healthcare record, including assessment of any active illness/condition and escalation to Duty MO if necessary
- On first admission commence the *ECT Multidisciplinary Comprehensive Care Plan* (ECT MCCP), which includes completing the patient comprehensive risk screening and assessment to inform individualised interventions for minimising patient harm. On subsequent admissions, review current ECT MCCP and confirm risks and plan information remains current.

Note: if any change to patient presentation or level of risk, a risk re-assessment and corresponding care plan must be completed. Changes must be communicated to the patient's home treating team and ECT treatment team.

Electroconvulsive Therapy (ECT) via Thymatron IV® Management CPS

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Admission procedure for ECT outpatients (day cases) cont'd

- A new MCCP must be commenced every three (3) months in line with the ECT consent process
- Complete admission NOCC and phase of care
- Check patient's Psychiatric Services Online Information System (PSOLIS)/webPSOLIS for:
 - Treatment, Support and Discharge Plan (TSDP) to ensure that it reflects the patient's ECT treatment, ongoing care and support
 - Risk Assessment and Management Plan (RAMP) to ensure that it is in date and accurately reflects current risks
 - Current What Matters Most document completed by the patient's treating team
 - If the above PSOLIS documentation requires updating the ECT admitting nurse is to ensure this is communicated with the home treating team and must ensure a copy of these documents are available in the patient's healthcare record.
- Complete a full set of physiological observations recorded on the *Adult Observation and Response Chart (AORC)*, and blood glucose level (BGL) if diabetic recorded on the *Insulin and BGL Record*
- Confirm medications to be withheld and escalate to ECT medical lead/anaesthetist if taken
 - All patients who may be required to take medication whilst on the ward will have relevant medication charts completed. If patient brings own supply, refer to the **Patients' Own Medication (POM) Use in Hospital Policy**
- Checks of clinical alerts, documenting and implementing of safety procedures as appropriate
- Checks of patient property and removal of any harmful items for safekeeping as per **RDBG Patient Property Management Policy** and **Potentially Harmful Items Management for Mental Health Patients SOP**
- Referral to **RDBG Respiratory Virus Infection Prevention & Management Clinical Guideline** for any respiratory illness specific screening and documentation.
- Complete the *Pre-ECT Treatment Nursing Checklist* (refer to [Appendix I: Pre-ECT treatment nursing checklist](#))
- Encourage patient to void prior to transfer to ECT Suite (to avoid urinary incontinence during procedure)
- Ensure the patient's medical record accompanies them to the ECT suite on every occasion
- Accompany the patient and support them throughout the procedure
- Provide clinical handover to ECT nurse coordinator noting relevant additional information (e.g. premedication, dentures).

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ECT procedural details

The *ECT Procedural Safety Checklist* ([Appendix II](#)) is installed in the ECT Suite in both treatment and recovery rooms. The checklist will be referred to at Sign In, Time Out and Sign Out and provides an overview of the pre-procedural safety checks, script prior to treatment and safety checks prior to leaving the ECT suite required by all team members during treatment and recovery.

ECT suite set up

The ECT Suite will be prepared by the ECT nurse and recovery nurse for intervention on the day of treatment, who confirms the following equipment is present and in working order:

- resuscitation equipment and defibrillator
- oxygen supply and suction, tubing, masks, self-inflating bag mask device
- back up oxygen cylinder including suction
- back up suction unit
- cardiac monitor and BP cuff
- pulse oximeter(s)
- ECT Thymatron IV® (or equivalent machine)
- sphygmomanometer, stethoscope, pen torch (for papillary assessment) and tympanometer
- sufficient stock of ECT and anaesthesia equipment is available
- medications (as prescribed, but usually include intravenous (IV) propofol as the anaesthetic induction agent, suxamethonium and other, neuromuscular blocking agents)
 - stock in the medication cupboard/resuscitation medications
 - suxamethonium and rocuronium (muscle relaxant) is stored in fridge until ready for use
 - sharps/pharmacological waste container readily accessible.

Emergency equipment (defibrillator, first line medication kit, oxygen and suction) is to be maintained, checked and situated within the treatment and recovery area.

Arrival to ECT

If ECT is required for a COVID-19 positive patient, they must be managed in the ECT suite as per the **R PBG Respiratory Virus Infection Prevention & Management Clinical Guideline**

On patient arrival to the procedure room the Escort nurse provides iSoBAR handover to ECT staff and all team members participate in the ECT Procedural Safety Checklist Sign-In. This should include verbal confirmation of consent by the patient (voluntary patients) which must be documented by the ECT medical lead.

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ECT procedural details cont'd

ECT procedure

- The patient is positioned supine on the procedure trolley with a pillow supporting the head. Shoes and socks are removed if required. Dentures should be removed as per anaesthetists' instructions, placed in a denture pot labelled with patient identification label and stored under the trolley with the patient's personal items.
- Staff should reassure patient throughout the procedure.

 Perform hand hygiene as per the **Hand Hygiene Policy**.

- Once patient is positioned and monitoring is attached (electrocardiogram [ECG], BP, oxygen saturations, EEG), the ECT medical lead will place EEG electrodes. The ECT Procedural Safety Checklist Sign-in and Time out is performed (including check of electrode placement) before ECT medical lead will perform ECT. Staff will support limbs during procedure. ECT Lead will confirm that appropriate seizure was induced and note when the seizure was terminated.

Post procedure

Post procedure nursing staff may need to assist turning the patient into recovery position if required by anaesthetist.

The ECT medical lead must document:

- seizure duration, noting whether motor or EEG, the quality of the EEG seizure, and quality of suppression of EEG seizure for every stimulus delivered
- the treatment on the treatment chart or record from the Thymatron IV® and any incidents in the patient's integrated notes.

The ECT nurse will complete the *ECT Nurse Documentation template* at Sign In and Time Out and prior to transferring patient to recovery area.

Transfer to Recovery Area

The anaesthetist will determine when the patient is ready for transfer to recovery room and will provide a handover to the recovery room nurse on transfer.

ECT procedural details cont'd**Recovery Area**

The ECT recovery nurse will supervise and support the escort nurse to care for the patient post ECT.

Recovery staff must:

- observe and monitor for signs of airway compromise
- initiate basic airway manoeuvres or use of airway adjuncts as clinically indicated
- monitor for potential adverse effects of procedure
- perform and document post-procedure observations on the recovery room section of the Anaesthesia record:
 - **every 5 minutes** until discharge from recovery
 - nursing staff will switch to use of the AORC for vital signs if the anaesthetist leaves the ECT suite prior to patient discharge or if there is no further room for observations on the recovery room record
- escalate to the anaesthetist any observations outside of normal parameters, the anaesthetist will review, and progress as clinically indicated
- communicate concerns with the treating team on the patient's recovery
- conduct the *Orientation questionnaire form* ([Appendix IV](#)) (provided by the ECT nurse in the suite and is to be completed by the escort nurse with the patient) 30 minutes post-treatment. The form must be completed before leaving ECT Suite
- ensure the *ECT Side-Effects Assessment Form* ([Appendix III](#)) has been added to the patients file for completion on return to ward
- document in the recovery and Sign Out section of the *ECT Nurse Documentation template on the DMR* prior to discharge back to ward area
- when all vital signs are stable, the intravenous cannula (IVC) may be removed prior to returning the patient to the ward. Refer to criteria in **PIVC Clinical Guideline** for ongoing site assessment.

Patient may be discharged back to the ward or unit when they meet discharge criteria – refer to [Operating Theatres NPS](#) – Appendix I: Recovery Discharge Criteria Checklist (direct link).

Patients should be escorted back to ward in a wheelchair.

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ECT procedural details cont'd**Nursing responsibilities on return to ward/unit**

The escort nurse is required to give handover to the patient's allocated nurse at time of patient return to ward. Refer to **Pre- and Post-Procedural Management NPS**

On return to ward the role of the ECT escort nurse is to:

- check patient's vital sign observations (and refer to *AORC* for escalation guides):
 - immediately on return to ward or unit
 - half hourly for one hour
 - hourly for 2 hours, thereafter
- complete *ECT Side Effects Assessment Form* ([Appendix III](#)) and escalate concerns to shift coordinator and medical team
- document in the patient's healthcare record and liaise with MO as clinically indicated. Activate MET as per the *AORC* criteria, if indicated.

Outpatients should ideally remain on the designated ward or recovery area for 4 hours post treatment and be discharged once the *Ambulatory ECT Criteria for Discharge* have been met, using the template in the patient's healthcare record. Patients from other sites can return with escort nurse once medically cleared for transfer.

Post-ECT treating team requirements**Inpatient course**

The post-intervention role of the treating team is to:

- document a summary of patient's condition post ECT, overall therapeutic outcome and adverse effects, including plan for continuation of course or planned end-point
- review any adverse effects experienced post-ECT, refer to the nursing *ECT Side-Effects Assessment Form* ([Appendix III](#)) to evaluate
- document plan for post-ECT clinical management and any plans for follow-up of adverse effects, including informing the ECT team to ensure correct reporting to the OCP
- review vital signs (nursing to document - treating team to review)
- review the notes from the recovery nurse, anaesthetist, or medical staff administering ECT, documenting occurrence and management of any complications during recovery
- review recommendations by anaesthetist suggesting any alteration in doses of anaesthetic agents and/or procedures for subsequent treatments (on anaesthetic chart)
- complete clinical reviews when required to discuss complex cases and the patient's treatment and progress with the ECT team and referring psychiatrist
- document any changes to the patient's MH legal status under the MHA 2014 between treatments and promptly advise ECT team to ensure patients are treated under the correct consenting requirements.

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Appendix I: Pre-ECT treatment nursing checklist

Please use I.D. label or block print

ROYAL PERTH BENTLEY GROUP Pre ECT Treatment Nursing Checklist WARD: _____ DOCTOR: _____		SURNAME				URN
		GIVEN NAMES				
		D.O.B.				GENDER
ECT Number	1	2	3	4	5	6
Date of treatment						
Medical Records Available						
Mental Health Act Status						
ECT Consent Form Valid						
ECT Treatment Form						
ECT Anaesthetic Consent Valid						
ECT Anaesthetic Consultation						
ID Bracelet X 2 in place						
3 point ID Patient Stickers Used						
Medication Charts in file						
Pathology results						
ECG Recordings						
A-ORC Chart – Ward Observations						
Baseline Observations Charted						
Weight / BMI						
Fasted from _____						
Time of last Void						
Not Smoked for 2hrs prior						
Hair Clean / Appropriate clothing						
Lipstick / Nail Varnish removed						
Jewellery removed or taped						
Valuables secured on ward						
Micro Alerts						
Pre-Meds given / time						
Prostheses (Specify)						
Dentures - IN / OUT / NA						
Dental - Chipped, loose, crowns						
Signature	Registered Nurse					
	ECT Nurse					

MR85.2
Pre ECT Treatment Nursing Checklist

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Appendix II: ECT procedural safety checklist

Government of Western Australia
East Metropolitan Health Service

Royal Perth Bentley Group

ECT Procedural Safety Checklist

Anaesthetist 	Anaesthesia Technician 	Treating Doctor 	ECT Nurse 	Recovery Nurse 
SIGN IN Condition: Before Sedation/Anaesthesia Initiated & led by: ECT Nurse Required:     +  1 Team Introductions Confirmed 2 Correct Patient & Procedure Patient name, DOB & UMRN Procedure name & consent Mental Health status Treatment number Electrode placement Essential imaging/investigations 3 Allergies 4 Infection Prevention Antibiotics indicated Temperature management 5 Bleeding & Clotting Anticoagulant use Thromboprophylaxis Blood loss risk (transfusion consent) 6 Briefings/Concerns Anaesthetic (monitoring, machine check, airway) Psychiatric or procedural Nursing (pressure care, bladder care) Pacemaker/other internal metal Pregnancy status Any other concerns 7 "Sign In Checklist Complete"				
TIME OUT Condition: SIGN IN Complete Before ECT Initiated & led by: ECT Nurse Required:     +  1 Any NEW Team Members 2 Confirm Patient & Procedure Patient name, DOB & UMRN Procedure name & consent Essential imaging/investigations Electrode placement and treatment percentage 3 Infection Prevention 4 Bleeding & Clotting 5 Script Prior to Treatment Patient is anesthetized, muscle relaxant given ECT can proceed ECT will be given now 6 "Time Out Checklist Complete"				
SIGN OUT Condition: Before patient leaves ECT Suite Initiated & led by: Recovery Nurse Required:   Escorting Nurse 1 Procedure Name 2 Specimens Labelled 3 Equipment Issues 4 Post-op Concerns/Instructions Bladder care Estimated blood loss Antibiotics Thromboprophylaxis Imaging/investigations Any other concerns 5 "Sign Out Checklist Complete" Always refer to SSC/PSC Policy + Guiding Principles				







All team members to stop and actively listen



Involve patients where possible

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Appendix III: ECT Side Effects Assessment Form

Please use I.D. label or block print

ROYAL PERTH BENTLEY GROUP		SURNAME		URN				
ECT SIDE EFFECTS ASSESSMENT		GIVEN NAMES						
		D.O.B.		GENDER				
WARD: _____		DOCTOR: _____						
Nursing staff to complete with patient – ask patient to rate (tick) the below side effects from 0 to 5 within one hour of returning to ward.				Time returned to ward:				
				Time assessment completed:				
Any side effect rated above 2 to be escalated for medical review.	0 (None)	1	2	3	4	5 (Worst)	Requested MO Review (tick)	
							Yes (note time)	N/A
Do you have a headache?								
Do you feel sick or have any vomiting?								
Are you feeling drowsy / sleepy?								
Do you have any muscle aches or pains?								
Are you feeling unsteady or having trouble moving around?								
Do you feel confused or dizzy?								
Nurse Name & Designation:				Sign:		Date:		



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ECT SIDE EFFECTS ASSESSMENT

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Appendix IV: Orientation questions

EAST METROPOLITAN HEALTH SERVICE MENTAL HEALTH	Surname		Sex	U.R. No
	Forenames		D.O.B.	
	Address			
	Ward	Registrar	Consultant	
	Use Patient I.D. Label when available			

ORIENTATION QUESTIONS																																					
(Test 30 minutes after each ECT treatment)																																					
☐ POST																																					
Date: _____	Admission Date: _____																																				
Time of ECT administration: _____	Time when orientation tested: _____																																				
Instructions:																																					
- Read out each of the 10 questions to the patient at <u>30 minutes</u> after the time of ECT treatment. Eg if ECT was at 8 am, test at 8.30 am - Tick the box if response is <u>correct</u> - Then provide feedback to the patient regarding whether the response is correct or not - Record the response verbatim for question 10, or if unclear if response is correct - For <u>question 10</u> circle most appropriate item (days or weeks) based on the admission date above. That is, if the admission date is <u>less than 14 days</u> before <u>today's date</u> ask for days, otherwise ask for weeks.																																					
<table border="1"><thead><tr><th>QUESTION</th><th>TICK if correct</th><th>Response</th></tr></thead><tbody><tr><td>1) What is your name?</td><td></td><td></td></tr><tr><td>2) What is your date of birth?</td><td></td><td></td></tr><tr><td>3) Who was the former prime minister of Australia?</td><td></td><td></td></tr><tr><td>4) Who is the current prime minister of Australia?</td><td></td><td></td></tr><tr><td>5) How old are you?</td><td></td><td></td></tr><tr><td>6) What year is it?</td><td></td><td></td></tr><tr><td>7) What month is it?</td><td></td><td></td></tr><tr><td>8) What is the name of the place (i.e., institution) you are in?</td><td></td><td></td></tr><tr><td>9) What day of the week is it today?</td><td></td><td></td></tr><tr><td>10) How many days / weeks* have you been in this place? *Circle whether you asked days or weeks (see above instructions)</td><td></td><td></td></tr><tr><td colspan="2">TOTAL CORRECT /10</td><td></td></tr></tbody></table>	QUESTION	TICK if correct	Response	1) What is your name?			2) What is your date of birth?			3) Who was the former prime minister of Australia?			4) Who is the current prime minister of Australia?			5) How old are you?			6) What year is it?			7) What month is it?			8) What is the name of the place (i.e., institution) you are in?			9) What day of the week is it today?			10) How many days / weeks* have you been in this place? *Circle whether you asked days or weeks (see above instructions)			TOTAL CORRECT /10			
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10) How many days / weeks* have you been in this place? *Circle whether you asked days or weeks (see above instructions)																																					
TOTAL CORRECT /10																																					
Name: _____																																					

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ORIENTATION QUESTIONS

PMR 79

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The following authors – or those holding the position at that time - will coordinate a full review of the literature relating to ECT practice in March 2027.

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Related policies, practice standards, clinical guidelines

RPBG Policy Hub

- [RPBG ECT HealthPoint page](#)
- [Pre and Post-Procedural Management NPS](#)
- [RPBG Anaesthesia Technicians Professional Standards and Scope of Practice](#)
- [Identification of Patients SOP](#)
- [Patients' Own Medication \(POM\) Use in Hospital Policy](#)
- [RPBG Patient Property Management Policy](#)
- [Potentially Harmful Items Management for Mental Health Patients SOP](#)
- [Respiratory Virus Infection Prevention & Management Clinical Guideline](#)
- [Hand Hygiene Policy](#)
- [Medical Emergency and Cardiorespiratory Arrest Management CPS](#)
- [PIVC Clinical Guideline](#)
- [Operating Theatres NPS](#)

WA Department of Health Policy Hub

- [The Chief Psychiatrist's Guidelines for the use of Electroconvulsive therapy in Western Australia 2006](#)
- [Department of Health Triage to Discharge' Mental Health Framework for SSCD](#)

Other

- [Charter of Mental Health Care Principles \(Mental Health Act \[MHA\] 2014\)](#)
- [Mental Health Commission's Consumer Handbook to the Mental Health Act 2014](#)

Related Forms

- *MH Assessment Form*
- *Relevant MHA 2014 forms*
- *Electroconvulsive Therapy Consent to Treatment form*
- *Risk Assessment Management Plan (RAMP)*
- *Pre- Electroconvulsive Therapy Treatment Nursing Checklist*
- *Electroconvulsive Therapy Referrers Checklist Sticker*
- *Physical Examination Form (SSCD)*
- *Patient Consent to Anaesthesia General or Regional Form*
- *MH Inpatient Admission Form*
- *Adult Observation and Response Chart (AORC)*
- *Insulin and Blood Glucose Level Record*
- *Multidisciplinary Comprehensive Care Plan*
- *Electroconvulsive Therapy Nurse Documentation Sticker*

Legislative requirements, the evidence and the standard

For information relating to the legislative requirements and standards that RPBG policy documents must adhere to, and regarding the logos and levels of evidence used within RPBG policy documents, refer to [Legislative Requirements, the Evidence and the Standard](#) (live link) on the Policy HUB.

Electroconvulsive Therapy (ECT) via Thymatron IV® Management CPS

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